

# Competitive Landscape: GLP-1 Receptor Agonists — Obesity & Type 2 Diabetes

**Segment:** Incretin-based therapies for chronic weight management and type 2 diabetes — injectable and oral  
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· 10 sections

**A note on this sample.** VigilLine is a healthcare competitive intelligence practice. This is a portfolio piece — a competitive landscape produced independently, entirely from public information, to demonstrate the depth and format of my work. It contains no client data and no employer-derived content.

**Why this segment:** my deepest independent work is in oncology. This sample exists to show that the method transfers — that the same monitoring, sourcing, and synthesis discipline applies to a different therapeutic area with a completely different commercial structure. The segment changes; the method doesn't.

## 1. Executive Summary

The incretin market changed structure twice in nine months. In December 2025 and April 2026, the first two oral GLP-1 therapies for chronic weight management reached the US market — Novo Nordisk's **Wegovy tablets** (oral semaglutide 25 mg, approved 22 December 2025) and Eli Lilly's **Foundayo** (orforglipron, approved 1 April 2026). Both launched at **\$149/month** self-pay, against a **\$245/month** Medicare net price for the injectables.[1][2][3][4]

That price convergence is the commercial event of the period, and it came from policy rather than from clinical data. A most-favoured-nation pricing agreement announced in November 2025 cut injectable GLP-1 Medicare pricing to **\$245 per monthly supply**, and the **Medicare GLP-1 Bridge** demonstration — live since 1 July 2026 — caps eligible beneficiaries' cost at a flat **\$50/month**. [5][6][7] For a class whose cash prices had exceeded **\$1,000/month**, this is a change measured in thousands of dollars per patient per year.[8]

Meanwhile the efficacy hierarchy has stratified cleanly by mechanism, and it now determines positioning more than brand does:

| Tier               | Mechanism      | Example            | Weight loss     |
|--------------------|----------------|--------------------|-----------------|
| GLP-1 mono-agonist | GLP-1          | semaglutide (oral) | ~13–17%[1]      |
| Dual agonist       | GIP + GLP-1    | tirzepatide        | ~21%[9]         |
| Combination        | amylin + GLP-1 | CagriSema          | ~20–23%[10][11] |

| Tier           | Mechanism              | Example     | Weight loss     |
|----------------|------------------------|-------------|-----------------|
| Triple agonist | GIP + GLP-1 + glucagon | retatrutide | ~28–30%[12][13] |

Three findings drive the outlook:

1. **Route of administration is now a competitive axis in its own right**, and it trades off directly against efficacy. The orals are cheaper and more convenient; the injectables and the late-stage pipeline deliver substantially more weight loss. Oral semaglutide's ~13.6% and orforglipron's ~12.4% sit well below tirzepatide's ~21% and retatrutide's ~28–30%. [1][2][9][12]
2. **Lilly holds the efficacy frontier and Novo holds the access position**. Retatrutide is the most potent agent in the class by a wide margin; Novo's CagriSema, however, **failed its head-to-head primary endpoint against tirzepatide** in REDEFINE 4 — a material negative for Novo's next-generation story. [11]
3. **The Medicare Bridge expires 31 December 2027**. The broader BALANCE model was delayed indefinitely in April 2026 after failing to reach 80% plan participation. Roughly eighteen months of subsidised access is the current visibility, and obesity pharmacotherapy is typically chronic — which makes the cliff a genuine demand-forecast risk, not a rounding error. [6][7][14]

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## 2. The Category: Why Incretins Matter

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- GLP-1 receptor agonists mimic glucagon-like peptide-1, a gut-derived hormone released after eating that drives insulin release, slows gastric emptying, and reduces appetite. Tirzepatide adds GIP agonism; retatrutide adds glucagon. [9][12]
- The class has moved from glycaemic control to a broad cardiometabolic footprint. Approved and studied indications now include chronic weight management, type 2 diabetes, cardiovascular risk reduction, obstructive sleep apnoea, knee osteoarthritis pain, MASH, and chronic kidney disease. [6][12]
- Scale: the US GLP-1 receptor agonist market was valued at **\$52.92bn in 2025**, with tirzepatide alone accounting for **\$27.14bn** — the single largest product segment. [15]
- The addressable population is far larger than the treated population. Lilly's chief executive framed it directly at the Foundayo launch: **fewer than 1 in 10 people who could benefit from a GLP-1 are taking one** — held back by access, cost, stigma, and perceived complexity. [2]
- That gap is the strategic prize, and it is why the oral formulations matter commercially even though they underperform the injectables on weight loss. Convenience and price expand the treated population; they do not necessarily capture share from existing injectable users.

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## 3. The Incumbents — Product & Company Profiles

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### 3.1 Semaglutide (Novo Nordisk)

|                         |                                                                                                                 |
|-------------------------|-----------------------------------------------------------------------------------------------------------------|
| <b>Franchise</b>        | Three distinct products: Rybelsus (oral, T2D, 2019), Wegovy injection (2021), Wegovy tablets (Dec 2025) [1][16] |
| <b>Wegovy tablets</b>   | Once-daily oral, 1.5 → 4 → 9 → 25 mg escalation ladder[1]                                                       |
| <b>Approval</b>         | FDA new dosage form approval 22 December 2025 (NDA 218316); broadly available 5 January 2026[1][16]             |
| <b>Pivotal evidence</b> | OASIS 4: <b>-13.6%</b> at 64 weeks vs -2.2% placebo; <b>-16.6%</b> in those who stayed on treatment[1]          |
| <b>Bioavailability</b>  | ~1–2% oral vs 89% subcutaneous — a ~50–90× absorption gap, addressed with the SNAC permeation enhancer[16]      |
| <b>Pricing</b>          | \$149/month starter (1.5 mg); \$199 for 4 mg after an introductory period; up to \$299 at higher doses[1]       |

**Strategic position:** Novo retains the deepest franchise — three products, one molecule — and inherited a cardiovascular risk-reduction indication for the tablet via pharmacokinetic modelling rather than a tablet-specific outcomes trial. That modelling basis is a differentiation weakness worth watching.[16]

### 3.2 Tirzepatide (Eli Lilly)

|                         |                                                                                                     |
|-------------------------|-----------------------------------------------------------------------------------------------------|
| <b>Franchise</b>        | Mounjaro (T2D), Zepbound (obesity), Zepbound KwikPen[6]                                             |
| <b>Mechanism</b>        | Dual GIP and GLP-1 receptor agonist[9]                                                              |
| <b>Weight loss</b>      | ~21% — the efficacy benchmark among approved agents[9]                                              |
| <b>Formulation note</b> | Medicare Bridge covers <b>KwikPen only</b> ; single-dose vials and single-dose pens are excluded[6] |
| <b>Indications</b>      | Weight management; obstructive sleep apnoea; T2D; cardiovascular risk reduction[6]                  |

**Strategic position:** tirzepatide remains the efficacy standard of care among approved products, and REDEFINE 4 confirmed it head-to-head against Novo's next-generation combination.[11] Its weakness is formulation fragmentation — the exclusion of single-dose vials from the Bridge creates a real access discontinuity for patients on that presentation.[6]

### 3.3 Orforglipron (Eli Lilly) — Foundayo

|                              |                                                                                                                                                                    |
|------------------------------|--------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| <b>Mechanism</b>             | Oral, once-daily <b>small-molecule (non-peptide)</b> GLP-1 receptor agonist — the first to market for weight management[2]                                         |
| <b>Approval</b>              | 1 April 2026, <b>50 days after filing</b> , 294 days ahead of the 20 January 2027 PDUFA date, under the FDA's Commissioner's National Priority Voucher pilot[2][3] |
| <b>Pivotal evidence</b>      | ATTAIN-1: <b>-12.4%</b> (-27.3 lb) at highest dose in those who stayed on treatment; <b>-11.1%</b> regardless of completion[2]                                     |
| <b>Dosing</b>                | 0.8 mg start → 2.5 → 5.5 → 9 → 14.5 → 17.2 mg maximum[4]                                                                                                           |
| <b>Formulation advantage</b> | Can be taken any time of day, <b>without food or water restrictions</b> — unlike oral semaglutide's fasting protocol[2][4]                                         |

|                |                                                                                            |
|----------------|--------------------------------------------------------------------------------------------|
| <b>Pricing</b> | \$149/month self-pay starter; from \$25/month with commercial coverage, via LillyDirect[2] |
|----------------|--------------------------------------------------------------------------------------------|

**Strategic position:** orforglipron is the strongest *commercial* launch in the class even though it is the weakest on efficacy. Three advantages compound: it is the cheapest approved GLP-1 in most payer scenarios; it carries no food/water timing restriction, which removes the adherence failure mode that dogs oral semaglutide; and as a small molecule it is chemically synthesised, which is materially cheaper to manufacture at scale than a peptide.[2][4][16]

## 4. Competitive Field — By Mechanism Class

### 4.1 Approved — GLP-1 Mono-Agonists (the legacy base)

| Molecule    | Brands                    | Route                          | Weight loss                 | First approval     |
|-------------|---------------------------|--------------------------------|-----------------------------|--------------------|
| Exenatide   | Byetta, Bydureon          | Twice-daily / weekly injection | ~3–5%                       | 2005               |
| Liraglutide | Victoza, Saxenda          | Daily injection                | ~8%                         | 2010 / 2014        |
| Semaglutide | Ozempic, Rybelsus, Wegovy | Weekly injection / daily oral  | ~14.9% (STEP 1, injectable) | 2017 / 2019 / 2021 |
| Dulaglutide | Trulicity                 | Weekly injection               | —                           | —                  |

**Strategic position:** the older mono-agonists are now commoditised and largely displaced in obesity by tirzepatide and the orals. They retain T2D formularies and a cost-advantaged position in some markets.

### 4.2 Approved — Oral Formulations (the new battleground)

| Product                        | Company      | Mechanism                    | Weight loss | Price        | Timing restriction                      |
|--------------------------------|--------------|------------------------------|-------------|--------------|-----------------------------------------|
| <b>Wegovy tablets</b>          | Novo Nordisk | Peptide GLP-1 (SNAC-enabled) | ~13.6%[1]   | \$149–299[1] | <b>Yes</b> — fasting/water protocol[16] |
| <b>Foundayo</b> (orforglipron) | Eli Lilly    | Small-molecule GLP-1         | ~12.4%[2]   | \$149–299[2] | <b>No</b> [2]                           |

**This is the sharpest competitive contrast in the class.** Two orals, launched within four months, at identical price points, with near-identical efficacy — separated almost entirely by *administration burden* and *manufacturing economics*. Oral semaglutide's ~1–2% bioavailability requires a 25 mg dose and an empty-stomach protocol; orforglipron needs neither.[2][16]

### 4.3 Late-Stage Pipeline — Next-Generation Mechanisms

## Retatrutide (Eli Lilly) — the efficacy frontier

- First-in-class **GIP / GLP-1 / glucagon triple hormone receptor agonist**, once-weekly.[12]
- **TRIUMPH-1** (NCT05929066, 80 weeks, obesity/overweight without diabetes): 12 mg delivered **–28.3% (–70.3 lb)**; **45.3%** of participants achieved  $\geq 30\%$  weight loss — a magnitude historically associated with bariatric surgery; **65.3%** of the 12 mg arm reached BMI  $< 30$ . In the 104-week extension, 12 mg reached **–30.3%**.[12]
- **TRIUMPH-2** (NCT05929079, T2D + obesity/overweight): 12 mg delivered **–20.8% (–49.6 lb)** at 80 weeks; **59.5%** no longer met the BMI criteria for obesity; up to 40% reached normal-range A1C. Published in *The Lancet*, presented at EASD September 2026.[13][17]
- **TRIUMPH-3** (severe obesity + established cardiovascular disease): both 9 mg and 12 mg met the primary endpoint.[13]
- **Filing:** Lilly is completing the CMC package for a BLA and plans submission in **Q1 2027**.[13]

**Strategic position:** retatrutide is the single most disruptive asset in the field. It is the only agent approaching surgical weight-loss magnitude, and it is being developed across obesity, knee osteoarthritis pain, OSA, chronic low back pain, cardiovascular and renal outcomes, and MASLD — a multi-indication expansion strategy that would be very hard to match on formulary.[12]

## CagriSema (Novo Nordisk) — the combination play, with a setback

- Fixed-dose combination of **cagrilintide (long-acting amylin receptor agonist) + semaglutide**, once-weekly.[10]
- **REDEFINE 1** (68 weeks, obesity/overweight): 2.4/2.4 mg produced **–20.4%** vs **–3.0%** placebo; 60% of participants achieved  $\geq 20\%$  weight loss.[10]
- **REDEFINE 4** (open-label, 84 weeks, n=809, head-to-head vs **tirzepatide 15 mg**): CagriSema 2.4/2.4 mg achieved **23.0%** vs tirzepatide **25.5%** (completers); 20.2% vs 23.6% (treatment-regimen estimand). **The trial did not achieve its primary endpoint** of demonstrating non-inferiority on weight loss.[11]
- **REIMAGINE 5** (T2D, head-to-head vs tirzepatide 5 mg, 60 weeks): CagriSema 1.0/1.0 mg achieved **–12.4%** vs tirzepatide 5 mg **–9.1%** — superiority met; A1C non-inferior (1.71% vs 1.67%).[10]
- **REDEFINE 9:** 1.0/1.0 mg produced **–21.0%** vs **–2.0%** placebo at 68 weeks.[10]
- **Readouts ahead:** REDEFINE 11 in H1 2027; a higher-dose CagriSema trial (2.4/7.2 mg) planned to initiate H2 2026.[11]

**Reading the two head-to-heads together matters.** CagriSema beat tirzepatide 5 mg (superiority) and lost to tirzepatide 15 mg (non-inferiority not met). That is a dose-comparison artefact as much as a mechanism result — but it is the comparison the market will price, and Novo's own framing leans on the REDEFINE 11 and higher-dose trials as the "full weight-loss potential" assessment.[10][11]

## 4.4 Oral Pipeline and Other Mechanisms

| Asset                  | Company      | Mechanism      | Stage             | Note                                                   |
|------------------------|--------------|----------------|-------------------|--------------------------------------------------------|
| Oral semaglutide 50 mg | Novo Nordisk | GLP-1          | Phase 3 (OASIS 1) | –15.1% at 68 weeks — <b>not the approved dose</b> [16] |
| Mazdutide              | —            | GLP-1/glucagon | Phase 2–3         | —                                                      |
| Pemvidutide            | —            | GLP-1/glucagon | Phase 2–3         | —                                                      |
| Survodutide            | —            | GLP-1/glucagon | Phase 2–3         | —                                                      |

**Watch item:** OASIS 1 tested oral semaglutide **50 mg** and reported –15.1%; the approved Wegovy tablet dose is **25 mg**, supported by OASIS 4 at –13.6%. The higher dose was not approved — which leaves headroom for Novo to escalate if orforglipron pressures the oral segment.[16]

## 5. Competitive Positioning Matrix

### By mechanism and efficacy

| Tier                               | Mechanism                 | Lead asset                          | Company         | Weight loss    | Status                                |
|------------------------------------|---------------------------|-------------------------------------|-----------------|----------------|---------------------------------------|
| <b>Tier 1 — Efficacy leader</b>    | GIP/GLP-1/glucagon triple | <b>Retatrutide</b>                  | Eli Lilly       | <b>~28–30%</b> | Phase 3 complete; BLA Q1 2027[12][13] |
| <b>Tier 1 — Approved benchmark</b> | GIP/GLP-1 dual            | Tirzepatide                         | Eli Lilly       | ~21%           | Approved[9]                           |
| <b>Tier 2 — Combination</b>        | Amylin + GLP-1            | CagriSema                           | Novo Nordisk    | ~20–23%        | Phase 3; missed H2H primary[10][11]   |
| <b>Tier 2 — Oral</b>               | GLP-1 peptide (oral)      | Wegovy tablets                      | Novo Nordisk    | ~13.6%         | Approved Dec 2025[1]                  |
| <b>Tier 2 — Oral</b>               | GLP-1 small molecule      | Foundayo                            | Eli Lilly       | ~12.4%         | Approved Apr 2026[2]                  |
| <b>Tier 3 — Legacy</b>             | GLP-1 mono                | Semaglutide injectable              | Novo Nordisk    | ~14.9%         | Approved 2021[9]                      |
| <b>Tier 3 — Legacy</b>             | GLP-1 mono                | Liraglutide, exenatide, dulaglutide | Novo, AZ, Lilly | 3–8%           | Displaced[9]                          |

### By commercial position

| Dimension                     | Leader       | Assessment                                                                                |
|-------------------------------|--------------|-------------------------------------------------------------------------------------------|
| Efficacy frontier             | Eli Lilly    | retatrutide ~28–30%, unapproached by any competitor[12]                                   |
| Approved efficacy             | Eli Lilly    | tirzepatide ~21%[9]                                                                       |
| Price leadership              | Eli Lilly    | orforglipron \$149 self-pay / \$25 commercial[2]                                          |
| Formulation convenience       | Eli Lilly    | orforglipron — no food or water restriction[2]                                            |
| Breadth of franchise          | Novo Nordisk | three products, one molecule; oral + injectable[1][16]                                    |
| Access position               | Novo Nordisk | Wegovy injection <i>and</i> tablets both in the Medicare Bridge; Zepbound KwikPen only[6] |
| Cardiovascular outcomes depth | Novo Nordisk | semaglutide has the longer outcomes record[9][16]                                         |

**The structural read:** Lilly leads on efficacy, price, and formulation convenience simultaneously. Novo's defensible positions are franchise breadth, the longer cardiovascular outcomes record, and a broader Bridge footprint. Novo's next-generation bet — CagriSema — failed its highest-profile head-to-head, which makes REDEFINE 11 and the higher-dose programme load-bearing rather than incremental.[10][11][16]

## 6. Pricing & Access — The Section That Matters Most Here

In most therapeutic areas pricing is a constraint on strategy. In incretins it *is* the strategy. This section exists because a competitive landscape in this class that omits access economics is not decision-useful.

### 6.1 The most-favoured-nation agreement

- Announced **6 November 2025** between the US administration and Eli Lilly and Novo Nordisk.[5][8]
- Sets a Medicare price of **\$245 per monthly supply** for injectable GLP-1s — Ozempic, Mounjaro, Wegovy, Zepbound — across all doses and indications.[5][6]
- Pre-deal list prices were approximately **\$1,349** (Wegovy) and **\$1,059** (Zepbound); the reported reduction is roughly **75%**.[8]
- Oral obesity pills priced at **\$149/month** across Medicare, Medicaid, and the federal direct-to-consumer channel, with higher doses up to ~\$399.[8]
- In exchange, manufacturers received a multi-year tariff exemption and committed to domestic manufacturing.[8]

### 6.2 The Medicare GLP-1 Bridge

| Parameter            | Detail                                                                                                                                                                                    |
|----------------------|-------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| Status               | Live since <b>1 July 2026</b> ; runs to <b>31 December 2027</b> [6][7]                                                                                                                    |
| Authority            | Section 402 demonstration — <b>outside</b> the Part D benefit, not a Part D plan offering[6]                                                                                              |
| Beneficiary cost     | Flat <b>\$50 per monthly supply</b> , any dose, any benefit phase[6][7]                                                                                                                   |
| Government net price | <b>\$245 per monthly supply</b> from participating manufacturers[6]                                                                                                                       |
| Central processor    | Humana (current LI NET administrator)[6]                                                                                                                                                  |
| Covered products     | <b>Foundayo</b> (all formulations), <b>Wegovy</b> (injection <i>and</i> tablets), <b>Zepbound KwikPen</b> only[6]                                                                         |
| Excluded             | Zepbound single-dose vials and single-dose pens; Ozempic; Mounjaro; Rybelsus; Saxenda[6]                                                                                                  |
| Eligibility          | BMI $\geq 35$ ; <b>or</b> BMI $\geq 30$ with HFpEF, uncontrolled hypertension, or CKD stage 3a+; <b>or</b> BMI $\geq 27$ with pre-diabetes, prior MI, prior stroke, or symptomatic PAD[7] |
| Critical exclusion   | Anyone already receiving a GLP-1 through Part D for T2D, moderate-to-severe OSA, or non-cirrhotic MASH is <b>ineligible</b> for the Bridge[6]                                             |
| Not applicable       | \$50 does <b>not</b> count toward the Part D deductible, <b>does not</b> count toward TrOOP or the out-of-pocket cap, and <b>no</b> low-income subsidy applies[6]                         |

### 6.3 The five strategic implications

1. **Price has been decoupled from efficacy.** Orforglipron at ~12.4% costs the same \$149 as oral Wegovy at ~13.6%, and the injectables sit at \$245 with ~21%. Payers will increasingly ask why they are paying for the efficacy premium.[1][2][9]
2. **The Bridge excludes the patients with the strongest clinical case.** Anyone whose GLP-1 is already Part D-covered — T2D, OSA, MASH, cardiovascular risk reduction — must stay in Part D and cannot access the \$50 copay. The Bridge is *only* for the weight-only population Medicare historically refused to cover. Two patients on identical Wegovy doses can pay very different amounts depending on the indication on the prescription.[6]
3. **The subsidy is a cliff, not a baseline.** The Bridge expires 31 December 2027. The intended permanent successor, the BALANCE model, was delayed indefinitely in April 2026 after failing to reach the required 80% plan participation. Obesity pharmacotherapy is typically chronic and stopping reverses the weight loss — so patients who build a treatment plan on \$50/month are exposed.[6][7][14]
4. **Formulation fragmentation creates access discontinuity.** The KwikPen-only restriction means a patient on Zepbound single-dose vials cannot get the \$50 price without switching presentation. That is an operational burden with adherence consequences.[6]
5. **Manufacturing economics now drive pricing power.** Orforglipron is a chemically synthesised small molecule; semaglutide is a peptide requiring SNAC-enabled formulation. At scale, the small molecule has a structural cost advantage — which is why Lilly can price at \$149 and still defend it.[2][16]

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## 7. Patent & IP Landscape

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| Player              | Position                                                                                                                                                                                                      |
|---------------------|---------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| <b>Novo Nordisk</b> | Semaglutide franchise IP across three products (Rybelsus, Wegovy injection, Wegovy tablets); the SNAC oral delivery platform is a separate formulation moat[16]                                               |
| <b>Eli Lilly</b>    | Tirzepatide (dual agonist) and orforglipron (small molecule); orforglipron was <b>discovered by Chugai Pharmaceutical</b> and licensed to Lilly in 2018 — a licensing dependency worth noting in diligence[2] |
| <b>Eli Lilly</b>    | Retatrutide — triple agonist composition claims; no approved competitor in the mechanism class[12]                                                                                                            |
| <b>Novo Nordisk</b> | Cagrilintide (amylin analogue) plus the semaglutide estate — the combination's defensibility rests on both components[10]                                                                                     |

**The orforglipron licensing chain is a genuine diligence item.** Lilly's most commercially successful new launch rests on a molecule discovered by Chugai (a Roche subsidiary) and licensed in 2018 — reported at \$50m upfront with up to \$390m in milestones. That is a very low price for an asset of this commercial scale, which suggests either favourable terms or undisclosed later-stage economics.[2]

## 8. Unmet Needs & White Space

| Unmet need                               | Current approaches                    | Gap                                                                   | White space                                                                           |
|------------------------------------------|---------------------------------------|-----------------------------------------------------------------------|---------------------------------------------------------------------------------------|
| <b>Maintenance after discontinuation</b> | Nothing approved                      | Weight regain is typical; no approved maintenance therapy             | Lowest-dose chronic maintenance; intermittent dosing regimens                         |
| <b>Muscle mass preservation</b>          | Body-composition monitoring only      | Weight loss includes lean mass loss; no approved muscle-sparing agent | Combination with myostatin inhibitors; resistance-training protocols as label content |
| <b>Oral efficacy parity</b>              | Orforglipron ~12.4%; oral sema ~13.6% | Orals deliver roughly half the injectable effect                      | Oral triple agonists; higher-dose oral formulations                                   |
| <b>Access continuity beyond 2027</b>     | Medicare Bridge (expires Dec 2027)    | No permanent coverage pathway                                         | Employer/plan contracting; cash-pay subscription models                               |
| <b>Tolerability / discontinuation</b>    | Dose escalation                       | GI adverse events are the dominant discontinuation driver             | Titration optimisation; co-therapies for GI tolerance                                 |
| <b>Adherence in orals</b>                | Fasting protocols (oral sema)         | Administration burden drives failure                                  | Non-peptide orals — the problem orforglipron already solved[2]                        |
| <b>Payer-defined value</b>               | Flat pricing by product               | No outcomes-linked pricing                                            | Outcomes-based contracts tied to weight/A1C/CV endpoints                              |

## 9. Strategic Outlook (Next 24–36 Months)

Catalysts that will determine the competitive order:

- **Retatrutide BLA submission, Q1 2027.** If approved, it is the first agent at ~28–30% weight loss — and the first to make the injectable-versus-oral efficacy gap unignorable.[13]
- **CagriSema REDEFINE 11, H1 2027.** Load-bearing for Novo's next-generation position after REDEFINE 4 missed its primary endpoint. The higher-dose 2.4/7.2 mg trial initiating H2 2026 is the second attempt.[11]
- **Medicare Bridge expiry, 31 December 2027.** The single largest demand-forecast risk in the class. Whether Congress or CMS creates a successor determines the 2028+ volume picture.[6][7]
- **BALANCE model restart.** Delayed indefinitely April 2026 at 80% participation threshold. Any revival is a positive for class-wide volume.[14]
- **Indication expansion.** OSA, knee osteoarthritis pain, MASH, CKD, chronic low back pain — each approval widens the Part D-covered population, which paradoxically *removes* patients from Bridge eligibility and shifts them to ordinary plan cost-sharing.[6][12]
- **Compounded alternatives.** Compounded semaglutide/tirzepatide pricing of roughly \$45–299/month continues to pressure branded pricing at the low end, with regulatory posture still unsettled.[5]

**Most likely trajectory:** The class stratifies into three durable segments rather than converging. **Efficacy** (retatrutide, tirzepatide) for patients and payers willing to fund maximum weight loss; **convenience and price** (orforglipron, oral semaglutide) for population-scale expansion where adherence and cost dominate; and **combination** (CagriSema) fighting for the middle unless REDEFINE 11 or the higher-dose programme changes its efficacy story. Lilly holds two of the three segments outright. Novo's defensible ground is franchise breadth, cardiovascular outcomes depth, and the broader Medicare Bridge footprint — a strong position, but a defensive one.

**The under-appreciated risk is the 2027 cliff, not the competition.** Every share model in this class implicitly assumes continued subsidised access. If the Bridge lapses without a successor, the volume assumptions behind the entire late-stage pipeline come into question simultaneously — which would hurt the most aggressive capacity builders disproportionately.

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## 10. Sources

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[1] Novo Nordisk — "Novo Nordisk's Wegovy pill, the first and only oral GLP-1 for weight loss in adults, now broadly available across America" (5 January 2026) — <https://www.prnewswire.com/news-releases/novo-nordisks-wegovy-pill-the-first-and-only-oral-glp-1-for-weight-loss-in-adults-now-broadly-available-across-america-302652205.html>

[2] Eli Lilly and Company — "FDA approves Lilly's Foundayo (orforglipron), the only GLP-1 pill for weight loss that can be taken any time of day without food or water restrictions" (1 April 2026) — <https://investor.lilly.com/news-releases/news-release-details/fda-approves-lillys-foundayotm-orforglipron-only-glp-1-pill>

[3] Life Science Daily — "Orforglipron Approved: Inside the Oral GLP-1 Race" — <https://lifesciencedaily.news/orforglipron-approved-inside-the-oral-glp-1-race>

- [4] The Daily Scout — "FDA approves Lilly orforglipron diabetes pill" (approval letter and dosing details) — <https://thedadaily scout.com/s/Q8MOGoRI8Uc>
- [5] Peptide News Digest — "TrumpRx and the January 2026 GLP-1 Price Cut: A Buyer's Guide to \$245 Wegovy, \$149 Pills, and Real Out-of-Pocket Costs" — <https://peptidenewsdigest.org/insights/trumprx-glp-1-price-cut-january-2026-medicare-medicaid>
- [6] CMS — "Medicare GLP-1 Bridge: Information for Part D Plans" — <https://www.cms.gov/medicare/coverage/prescription-drug-coverage/medicare-glp-1-bridge/information-part-d-plans>
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## Sources and Method

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**This analysis was prepared exclusively from publicly available information.**

| Source class              | Examples used                                                           |
|---------------------------|-------------------------------------------------------------------------|
| Regulatory and government | CMS (Bridge documentation, FAQ memorandum, press releases)              |
| Company disclosure        | Eli Lilly investor relations; Novo Nordisk announcements                |
| Peer-reviewed literature  | <i>The Lancet</i> (TRIUMPH-2, via PubMed); OASIS trials                 |
| Trial registries          | ClinicalTrials.gov (NCT05929066, NCT05929079, NCT06534411, NCT06388187) |
| Industry and trade press  | BioPharm International, Pharmacy Times, Life Science Daily              |
| Market analysis           | Kings Research US market sizing                                         |

**No non-public, confidential, or employer-derived information was used or solicited in the preparation of this analysis.** No primary research, expert interviews, or insider sources were used. Where a claim rests on a single source, or where trade press is reporting a company announcement rather than a primary filing, it is cited as such. Primary regulatory and company sources are preferred throughout; the CMS documentation and company investor-relations releases carry the load in Sections 6 and 4.3.

**Method:** continuous monitoring across the six source classes above, triage for materiality, trial-level analysis, competitive positioning, and decision-ready synthesis. Every material claim carries a numbered citation — 22 sources in total.

*This deliverable is competitive intelligence prepared from public sources. It is not legal, regulatory, medical, investment, or tax advice. Pricing and access terms in Section 6 are policy-dependent and subject to change; the Medicare GLP-1 Bridge is a time-limited demonstration scheduled to expire 31 December 2027.*

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